

Harold Brem, MD FACS
Professor of Surgery
Stony Brook University School of Medicine
Chief, Division of Wound Healing & Regenerative Medicine
Winthrop University Hospital
120 Mineola Avenue
Mineola, NY 11501
hbrem@winthrop.org
www.complexwoundhealing.org

Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for “Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments.”

Dear Sir or Madam:

My name is Harold Brem, and I am a general surgeon, Professor of Surgery at Stony Brook University School of Medicine and Chief of the Division of Wound Healing and Regenerative Medicine at Winthrop-University Hospital. My team of physicians and surgeons perform over 1000 operations with regenerative medicine products per year, 24/7, from simple ulcers to the aggressive necrotizing fasciitis. I am pleased to submit the following comments on two of the Draft Guidances for Industry and FDA Staff entitled: “Homologous Use of Human Cells, Tissues, and Cellular and Tissue-Based Products” and “Minimal Manipulation of Human Cells, Tissues, and Cellular and Tissue-Based Products.”

I would like to begin by applauding the Agency for its issuance of these draft guidance documents which will bring much needed clarity to industry – and thereby create certainty for clinicians that the human cell- and tissue-based products we use to treat our patients are safe and effective.

The tiered, risk-based approach embodied in the existing regulatory framework is entirely adequate - when complied with - for determining whether a product is appropriate for regulation solely under the Section 361 pathway, rather than needing premarket the demonstration of the product’s safety and effectiveness. However, today there is a vast array of new allograft-derived products

on the market without proven efficacy. Many of these products make a range of therapeutic treatment claims that involve complex cellular and biochemical interactions with the body that for any other product type would require FDA pre-market review, commensurate with their risk level. It is clear that allograft products that make claims about their cellular activity should be regulated as biologics.

Arguments in favor of the status quo, which allows allograft distributors to evade the need to generate valid level-one evidence that has been subjected to rigorous peer and regulatory review by FDA, put patients at risk. Contrary to the assertions of many in the allograft industry, it is not the case that imposing premarket review requirements will delay or prevent the entry of important therapies. Investment funding is well-available for promising biotechnology, and alternative pathways currently exist for addressing unmet clinical needs through accelerated review. Furthermore, the FDA should be perceived as a partner to doctors, patients and industry in working with sponsors to help bring safe, efficacious and high quality products that patients deserve.

The remainder of my comments address the minimal manipulation and homologous use draft guidances specifically. In order ensure that the tiered, risk-based framework outlined in 21 CFR 1271.10 functions properly, there must be clearly defined boundaries, which these guidances for the most part accomplish.

First, regulating cells and tissues based upon their primary or main effect not only provides administrative efficiency but provides certainty to the regulated industry. This principle is well established and consistent with FDA's approach for its regulation of biologics, drugs and medical devices and is entirely appropriate for human cell- and tissue-based products.

Second, the distinction contained in both the minimal manipulation and homologous use guidance documents between structural and non-structural tissues and cells is long-standing and entirely appropriate. FDA has previously explained its reasoning for this distinction, which is that structural tissues raise fewer safety concerns beyond adverse local effects. I urge the FDA to articulate more fully the rationale for preserving this distinction when it finalizes the guidance documents as doing so will assist industry and other stakeholders in applying the criteria of section 1271.10.

Third, with respect to tissues that can serve both structural and nonstructural functions, I believe that the approach taken in the minimal manipulation guidance document, referring to "the main

function of the human cell or tissue product, in the donor” is appropriate and should be preserved when the documents are finalized. The minimal manipulation criterion focuses on the particular purpose to which a tissue will be put. It is therefore entirely logical to determine the category into which the tissue falls by focusing on its main or primary function *in the recipient*. This approach dovetails appropriately with the homologous use guidance document, which relies on the primary intended effect to determine homologous use. I urge FDA, however, to articulate more clearly when it finalizes that guidance that the use of a non-structural tissue for structural purposes or vice versa would not constitute a homologous use.

In conclusion, I join other commenters who have urged FDA to move swiftly to finalize these guidance documents. Imposing regulatory order in the wound healing space is critical to protecting a particularly vulnerable, chronically ill patient population who deserve therapies that are proven through valid scientific evidence.

Thank you for the opportunity to comment on these guidance documents.

Sincerely,

Harold Brem

Harold Brem, M.D.

